

UCC/HSE: What Changes and What Does Not Change

What Changes

ACA premium-linked subsidies end. Federal support shifts to bounded catastrophic credits and formula-based HSE deposits.

Routine care moves out of comprehensive premium financing. Primary care, routine labs, imaging, behavioral-health visits, chronic monitoring, and other predictable services are financed through HSE accounts, targeted federal liquidity support, and transparent direct-pay markets.

Catastrophic risk becomes the insured layer. Major hospitalization, trauma, severe acute illness, surgery, ICU care, and other high-cost events remain pooled and protected.

Employer tax policy changes. The employer exclusion phases down; employers may contribute to HSAs, facilitate catastrophic enrollment, or shift compensation into wages.

ACA exchange architecture winds down. Metal tiers, CSRs, actuarial-value complexity, and income reconciliation are replaced by standardized catastrophic plans and HSE deposits.

Routine-care price discipline increases. HSE encourages transparent pricing, direct primary care, independent labs, imaging centers, ASCs, and efficient outpatient markets.

Rural providers receive targeted stabilization. A permanent rural access-capacity payment and temporary transition floor protect low-volume access providers.

Benefit-creep guardrails apply. Supplemental coverage cannot recreate first-dollar comprehensive insurance with federal support.

What Does Not Change

Medicare is unchanged. No change to eligibility, traditional Medicare, Medicare Advantage, Part D, or Medigap.

Medicaid LTSS, ABD, and HCBS are preserved. Medicaid remains focused on long-term services and supports, disability-related care, and home- and community-based services.

CHIP and medically complex children are protected. Pediatric HSE support coordinates with, but does not replace, existing pediatric safety-net programs.

Emergency-care obligations remain. EMTALA screening and stabilization duties are not altered.

Private insurance remains. Insurers may offer UCC-compliant catastrophic plans and limited supplemental products.

State roles remain. States retain solvency regulation, consumer protection, Medicaid coordination, rural supplements, and implementation functions.

Chronic patients remain protected. Chronic-condition support is provided through enhanced financing beneath a uniform catastrophic benefit.

The catastrophic plan remains uniform. Vulnerable groups receive additional financing, not separate second-tier insurance.

Bottom line: UCC/HSE is not repeal-only, not single payer, and not a pure HSA expansion. It is a replacement architecture: universal catastrophic protection for citizens/eligible individuals, prefunded routine-care accounts, preserved Medicare and core Medicaid functions, rural-provider stabilization, private-market participation, and bounded federal liability.